

I: Yeah.

I: Okay.

I: So then we're gonna start with the actual questions.

I: This one is about familiarity with AI.

I: So how familiar are you with AI technologies in general?

P: I mean, I'm just a user of AI tools, especially to search.

P: Just similar of our using in Google.

P: So we search for something.

P: The difference is this copilot.

P: So if you ask for something, he can create that more

P: customized to what you really need, like article, scientific articles, or restructuring some texts that you want to use to communicate in social media, let's say, more like, like a guides, not not to do the job, let's say.

I: yeah um could you also give a score of one to ten how familiar you think you are so one being low and ten being high

P: i don't think i am really familiar i only use chat gpt and the copilot is already installed in our computer so i would say a four

I: okay

I: Um, okay.

I: So you said you, you're using ChatGPT and also Copilot.

I: So you said you, you work with different, um, you do freelance, so you work with different companies, right?

I: Yeah.

I: Yeah.

I: So, um, do you use Copilot in all of those organizations or is it one, one company allows you to use it and one company doesn't?

P: I don't use copilot or AI tools on my practical.

P: I use as a tool to communicate in social media, let's say, or to make a research for my study.

P: Let's say I don't use AI on my work.

I: Yeah.

I: So you don't.

I: Yeah, you don't use it in treatment, but you do use it in, for example, just doing checkups like like you yourself are still studying or.

I: You're not in university, but you're still learning more every day, right?

P: Yeah, exactly.

P: In my study, in my going deep to my knowledge, let's say, I'm remembering that I use ChatGPT in one context specifically, that it's about co-pilot, that it's about searching for concepts.

P: the meaning of some concept because in some exercises we should know we ask the person to know what means loyalty or whatever okay and sometimes people don't don't know how to do that they don't know the meaning or say the meaning so we asked co-pilots for this concept and then we

P: work from this definition let's say but it's in um it's kind of uh in a way of joke because sometimes people brings us this um this uh behavior of um i i also asked chat gpt but he didn't say much yeah so let's talk chat gpt what means thing yeah yeah

P: And sometimes I use in that context, let's say.

I: Yeah.

I: Okay.

I: So you have used it one time, right?

I: Yeah.

I: And in that case, it was mostly a translation tool, right?

I: Yeah.

I: Is it that they didn't understand the term in Portuguese or was it in English?

P: No, even in Portuguese, for example, what means loyal?

P: What is loyalty?

P: Or what does it mean being honest?

P: Or what's the difference between forgiving and forgetting?

I: And then you would just ask ChatGPT that or Copilot and it would just give some...

P: I invite people to ask ChatGPT and then we discuss if the definition makes sense for the person.

I: Yeah.

I: Okay.

I: So your organization itself has not introduced any AI tools, right?

I: Like formally.

I: Yeah.

I: Okay.

I: Um, and in those cases that you did use, uh, AI, you did tell them, uh, that you were using AI, right?

I: Because you, you kind of use AI with them.

I: So they already know that you're using it.

I: Yeah, exactly.

I: Okay.

I: Um, then I think we can move on to the next question.

I: This is about diagnostic and assessment use.

I: Um, what are your thoughts on AI being used in diagnostic or assessment processes?

P: um it's a tricky question because in what i feel even talking with other colleagues in the beginning of their careers for example it can be too easy to ask chpd to interpretate some results of evaluations clinical evaluations or to

P: to build this model concept to interventions.

P: And in psychology, at least, each intervention plan is unique.

P: It should be unique.

P: You can have different sources, different models of intervention,

P: different techniques and you make a combination of all of that and you put in order in some order that you will use according to the person that you have, according to the needs that this person is bringing in that moment.

P: So it's very specific.

P: Also, there is this

P: um this in this sensation this thing that i i have um um noticed that chp it's the only platform that i'm i know until now uh it's more it's a pretentious like he's he goes to

P: in order to confirm your beliefs or to confirm what you are bringing.

P: And not only this works in a therapeutic process, okay?

P: There is something in therapeutic process that we need to pay attention, that it's about if the way

P: person is talking about the situation it's actually what they are feeling about the situation so you have this um not match between what they say and what they are doing or what they are expressing and this is not so this um divergence it's not the same in everyone

P: You need to know the person to see what micro-expressions can be activated in their faces or in their bodies, that it's from the person combined with what the person is saying.

P: So it's this kind of detail.

P: I confess that I already forgot the question.

I: That's okay.

I: So the question was, what are your thoughts on AI being used in diagnostic or assessment processes?

I: So you mentioned a few things, right?

P: Okay, so all of these to say what?

P: All of these to say what?

P: I think it can be a research tool to check on some details, to check on some differential diagnosis in the sense of

P: to check if this symptom is common in other pathologies, but never to confirm the diagnosis.

I: Okay.

I: Okay, that makes sense.

I: So kind of to summarize, right, what you mentioned is that the...

I: There is some kind of mismatch between what people are saying and what they're actually feeling, right?

I: So sometimes there might be some more qualitative data.

I: So for example, someone is touching their face or some like whatever thing is happening and AI cannot really see that, right?

I: So there is a mismatch between the data that AI will get and the data that a person might get.

P: Right.

I: And then also you mentioned that it might jump to conclusions and it also has this kind of confirmation bias.

I: Right.

I: So if you give it some information, it might tell you, oh, it's this or you're totally right.

I: It is anxiety disorder or whatever.

I: But maybe if you look at it on a personal level, there might be some deeper cause for some anxious symptoms or whatever, right?

I: And maybe as a person, you would not jump to that conclusion that fast, but AI is tending to be like, oh, it's anxiety disorder, right?

P: Yeah, I can give you examples.

P: For example, in some personality traits, personality standards or patterns,

P: you can have anxiety symptoms or depression symptoms or even avoiding symptoms and this doesn't mean that the person has depression or has anxiety or has a problem in their relationships there is a context when where these symptoms appear and there are triggers in these contexts that

P: for you can be trigger for some symptom and for me can be another one.

P: So it's so specific.

P: There is a joke, a general joke, I don't know if it's global joke or just in Portugal, that psychologists, when you ask something to a psychologist, they always answer you with, it depends, because it is, you know, it's really...

P: specific in each case.

P: Yeah.

I: So it's very context dependent also.

I: So, for example, for you, a trigger might be that whatever.

I: And then for me, the same trigger or the same external motivators wouldn't result in anything.

I: Right.

I: I wanted to check one more thing.

I: So you said you could use it for research tool and to do like differential diagnosis.

I: Right.

I: But you said to not confirm a diagnosis.

I: Do you mean that you shouldn't ask ChatGPT or Copilot or whatever AI system like, oh, this is my diagnosis.

I: Is this correct?

I: Do you think that's a wrong use?

P: I think it's a wrong use.

I: Yeah.

P: I think it's a wrong use.

P: The same way when we ask Dr. Google, for example.

P: Also, because this thing that I told you before, these pretensions, they tend to confirm, to follow your thoughts, to validate your thoughts.

P: And not all the thoughts should be validated.

P: So, according to the diagnosis, it's usual

P: people get a diagnosis and then they will search for what does it mean, what means this in relationship, in work, in whatever.

P: Just to understand better my point of view, not only

P: It's not only the best option to share the diagnosis with a person.

P: You should share, but the elements that confirm the diagnosis goes far away from the criterias of the DSM, for example.

P: You can have this checklist on the manual and you have all of that and still it's not a disorder.

P: Because of the context, because of what motivates these symptoms or how long they have these symptoms and whatever.

I: Yeah, so the DSM has a lot of criteria for specific disorders, right?

I: So you're saying that sometimes maybe someone ticks a few boxes, but that doesn't mean necessarily that that is a disorder because it might not be a problem in their life or not a problem for anyone around them.

I: So then it's not necessarily...

I: a problem that should be addressed, right?

I: Like maybe there is some parts or maybe some symptoms that you want to work on, but it doesn't necessarily mean that someone has a depression or that there is like a very strict way that we need to diagnose them.

I: Yeah.

I: Yeah.

I: Okay.

I: So can I also assume that the, for example, the intake processes and preliminary screening, you also don't think AI would be useful there?

P: I don't know if I understand the question.

I: So the intake, I think you're doing intakes as well with your clients, right?

I: Where the first session with someone, there's an intake, you ask them a lot of questions and you try maybe not necessarily to diagnose, but try and figure out what's going on.

I: And also maybe even before the intake, you might do screening.

I: So give them a questionnaire or...

I: ask them questions beforehand.

I: You also think that AI cannot be useful there in those areas?

P: It could be useful in the way that AI

P: has a as a an application or as a program that we can use to process the data okay in this kind of questionnaires it can be really useful but they cannot be the one which will interpretate the data

P: They can also not be the one who will search for this anamnesis, this background from the person, this intake.

P: Because according to what the person says,

P: you can follow your interview, your structure interview, or you can go around or you can notice something that triggers you and you go from this way to this way.

P: But also in this, how could we use, could you repeat the question?

P: Because I would like to say something more.

I: Yeah.

I: So I added two questions, which is what about the intake processes and have you considered AI for preliminary screening?

I: So the screening beforehand or during the intake, do you think AI could be used for that?

P: Yeah.

P: So when we use these structural evaluations like questionnaires, not all the approaches uses this kind of tool.

P: And not always we use this on the beginning, in the beginning of the process.

P: Not in every process we use the same questionnaires.

P: And what I would like to say more about this is how dangerous can be a person knows their results without context.

P: So for AI platforms having access to these kind of tools,

P: can be really dangerous.

P: These tools are used in setting, which means we in session, we use only in session and it's confidential and you cannot share with people outside the question because of the confidentiality.

P: So,

P: How could we guarantee that there will not be a bug that share this with people or a hacker that has access to that?

P: In computer, when we make our registrations, our model concepts, et cetera, we never work

P: the name of the person because of these hackers and things.

I: So currently in your job, if you make any report or whatever, you also try and not include that much personal information for data privacy stuff.

P: Yeah, data privacy and all the information that we produce to, for example, to recommend the psychiatrists or to send to a school, for example, to a professor, all the content in the document as client consents.

I: Yeah, yeah.

I: Yeah.

I: So how do you currently have the connection between the actual client and the documents then?

I: Because if you cannot use a name, like is there some identifying like a number or like how is this connected?

P: A code.

I: Yeah.

I: So there's just some place in your organization that has their, their names and, and some code and that code corresponds with a report or like.

P: It can be, it can be initials, um, the initials like [PERSON ONE], uh, [P1] for example.

P: Um, but they are not in the, uh, in the clinic.

P: They are.

P: on the therapist.

P: I'm the one who is responsible for the data.

P: So I am the one who creates the codes.

I: So it's not like 100% safe, right?

I: Because maybe if someone steals from you, then they will have that data, right?

I: But

I: there is a lot of steps or a lot of things that you put in place so there is um like very unlikely that all of those steps because even if someone gets the data from you it doesn't mean that they necessarily can say our queue is is this person right and this is useful even in supervision

P: We have supervision.

P: It's a meeting that we have with other colleagues to get inputs on this intervention.

I: Like second opinion, right?

P: Like a second opinion.

P: And even in these cases that the confidentiality is guaranteed between us,

P: Even in that cases, we don't share the name or the personal details.

P: Yeah.

I: Okay.

I: Makes sense.

I: Yeah.

I: Okay.

I: I think we can move on to the next question.

I: This is about monitoring and progress tracking.

I: How do you view the role of AI in tracking patient progress over time?

P: How can I imagine AI in monitoring client process, right?

P: I mean, there are some apps that already make this monitorization.

P: of humor, some exercises.

P: But I cannot imagine how AI could monitoring these some symptoms or some achievements in the therapeutic process.

P: Maybe because I don't know enough about AI, but I cannot imagine a way.

I: Yeah.

I: I mean, I also don't want to stare you in any way, right?

I: But maybe if you could explain a bit about how monitoring is done currently.

I: So do you do any monitoring of your clients or how do you track if they're going the correct way or if their symptoms are getting worse?

I: Like, how do you do that?

P: Yeah, we have we have this behavioral behavioral observation, which means that in each session, we check on the symptoms, and they are

P: different kind of symptoms they are these emotional symptoms that people currently knows know about anxiety anxiety for example we can check on how many panic attacks how many difficulties of regulation the breathing etc but there are also other kind of symptoms so according to

P: the map that we build for example in the beginning of the process of what kind of symptoms it's typical in this person when they experience anxiety or sadness we can check how often they have

P: these symptoms in this week or in this month or how effectively was the tool that we have to help to regulate the breathing for example or how is their sleep and how many for example in OCD how often you check on the things how often you need

P: You have this need to clean the things.

P: So we can make an app where we can create these kind of schedules.

P: But honestly, I don't know if the app it's

I: integrate on ai because people already do apps before ai so i think it's not the same right

I: yeah okay so as i understand it you in the beginning of the the treatment you will identify some symptoms right and you will track these over time so every session you just make observations of someone

I: And there are some very factual observations.

I: So maybe, oh, I slept this many hours tonight or whatever, right?

I: But there is also maybe some more subjective observations.

I: So for example, if your client would say, oh, I'm feeling really sad, like there's not a...

I: there's not a number for really sad, right?

I: So, but there's still an observation for that.

I: So you could mention it in like some tracking or in some report, you just write it down.

I: Okay.

I: Feeling more sadness this week.

I: And maybe there's some trigger for this or whatever, but currently what you're doing is just, you do the sessions with your clients and then during the sessions you write down, okay.

I: Oh, they mentioned they slept bad or they are feeling worse or,

P: Yeah.

P: And usually we correlate this to some episode or something that the person is dealing with.

P: You were talking and I was remembering.

P: I forget.

I: Remembering and then forgetting.

P: Yeah.

P: I was thinking to say something.

P: Because you were talking about this monitoring.

P: I don't remember.

I: Maybe I don't remember.

I: That's fine.

I: Let's see.

I: So how would you feel about AI taking on monitoring tasks in your practice?

I: You kind of don't feel like there is much space for this, right?

P: Yeah, I cannot imagine a way that AI could help us monitoring that.

P: That it's not already done in other platforms.

I: Okay, then we can move on to the next question.

I: This is about administrative and practical support.

I: How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

P: scheduling reminders you said okay i think this this was the thing that i was thinking about that i forget and now i think i remember again uh about the scheduling it could be nice for a small employer small entrepreneurs

P: um to have this assistant in scheduling let's say um but again i'm not sure if this already exists like a nona or a book or yeah yeah

P: Can you repeat the question just to check if I forgot something?

I: So the beginning of the question is, how do you feel about AI generating drafts or reports?

I: And then the rest was, or assisting with scheduling and reminders, but the scheduling and reminders you answered, but then for the, do you think there's any place for AI to be writing down the summarizing of sessions or making a report or making drafts or reports like?

P: I can only imagine about reports, for example, maybe helping to organize information, let's say.

P: But usually people build their own structure, report structure.

P: I mean, this is an example that could work on this guide or suggestion tool.

P: I would like to make this, to report these results.

P: How should I communicate them?

P: And they find

P: some examples of reports and say usually this kind of results that we report like that this kind of tool but this is a this is a help that i only imagine being useful for beginners yeah yeah i feel that my suggestions are

P: mostly related to study um help in our professional context yeah so what i'm seeing is that um

I: For most of the things you're saying, right, the actual idea of AI taking over tasks is not really relevant or possible, but it might be able to support in some kind of way that's more supportive.

I: So for example, with the report writing,

I: it shouldn't take over the actual report writing, but it might be useful to use it and ask, hey, how should I be writing this report, right?

I: And that is also more useful for people that are just starting out where they don't have any knowledge or like experience in doing this as much.

I: And like someone that has more experience already has their own technique or their own way of writing down the report.

I: So they won't really need it.

P: For example, I already have my report structure, my notes structure, the way I do.

P: And it's changeable in the sense that if I do some course or if I do some

P: Formation, let's say, I can learn something more and can make sense for me to add this section in my report structure, for example.

P: So it's really...

P: There is a big challenge in our profession that it's... Because it's so customized, our work,

P: it's really difficult to have access to examples or to check, okay, to this we do that, to this we do that.

P: We cannot do this in our profession.

P: So in the beginning can be really hard to feel confident about some communications documents.

P: That's why I can see

P: better, let's say, how useful could be in this kind of situations.

P: But there is always this but, that is, but you cannot consider this for everything.

P: It's just a guideline.

P: It's just a suggestion.

I: Yeah, so it might be useful to come up with some suggestions or some ideas, but it's not necessarily always true, but it could be used to try and be a bit creative.

I: Maybe, oh, it does give me this layout or this structure.

I: Hey, maybe I take this part and that is useful for me.

I: um but then again also because psychology is so very context dependent and person dependent and it's it's dependent on the person the client and also dependent on the psychologist right so everyone has their own style and i think what research shows is that it's most important how the connection is between client and psychologist that's um

P: that shows like the the benefit of therapy right yeah and not necessarily how therapy goes right it's mostly the connection so it's about relationships and every jury everything it's about relationship and the relationship it's the the base when everything happened yeah

I: Uh, yes.

I: Okay.

I: Um, let me see.

I: I think we can move on to the next question.

I: This is about psychoeducation and between session tools.

I: What are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

P: Psychoeducation.

P: I cannot let go away this thought that somehow AI can be an upgrade of Dr. Google and it can be really useful if the source are real and

P: and correct.

P: But I have experienced it with other colleagues that this psychoeducation that AI does, it's not only always correct and really useful, you know?

P: So I'm really skeptical of how much risk has this possibility between tasks, between sessions.

P: It could be nice to create the tools

P: so and this can be a difference between other platforms or other digital sources that is the to create the the documents you know like okay i would like to make a table to check some kind of thoughts or to monitoring some kind of symptoms okay and

P: the AI produces your document.

P: So you don't have to go there on Word or Excel or things.

P: But again, for this pinky layout, I use Canva.

P: So I'm not sure if Canva is AI.

P: Maybe I'm not a four, maybe I'm a three.

I: I just had a discussion today with my supervisor and I don't think you actually want to change it but we discussed that some I've had one psychologist after the conversation that we had also

wanted to change her familiarity score down but I would tell you that we shouldn't be doing that because

I: The goal of the study is just to figure out how familiar do psychologists think they are and not how familiar they think they are after we discuss a lot of things, right?

I: So I will keep it the same, but now you at least understand why I want to keep it that way.

I: Yeah, so what you're saying, you don't really see that many opportunities here for psychoeducation and between session tools, right?

I: yeah yeah um let me see i do want to ask like a few more questions on this so i don't think yeah would you

I: Yeah, so what you mentioned is mostly the between session tools for psychologists, but not necessarily for clients.

I: Or like maybe you mentioned it a bit, but you also don't think it would be beneficial for clients to have some AI tool that they could interact with or that they could use between the sessions that they have with their psychologist?

P: No, I don't think so.

I: Mm-hmm.

P: I cannot imagine in a way that this could be helpful.

I: Yeah.

I: Okay.

I: Makes sense.

I: Then let's move on to the next question.

I: This is about risks and limitations.

I: What risks or concerns do you have about using AI in clinical work?

P: What risks I'm concerned about using AI

P: freely to discuss our emotional things, issues.

P: It's the perception that people build of how shallow can be this comprehension.

P: I will explain.

P: It's been really often

P: people appears with some rational thought of why do i avoid romantic relationships because my dad was that because my dad did something and this can not only be wrong

P: but also compromise the therapeutic process because people build some belief around this thought and it can put more in the problem instead of give them a real comprehension.

P: Okay.

P: You know, people connect with this kind of impression.

P: Even this week, I have this girl who said, I know I have daddy issues.

P: Yeah.

P: And how do you know that?

P: TikTok.

P: Okay.

P: Oh, right.

P: Okay.

P: So how TikTok pretend to solve that?

P: Okay.

P: And why do you think you have daddy issues?

P: ah, because that, because this, something was right, something was wrong, but it's not always a good input on the therapeutic process.

P: Also because the therapy, it's not a place just to understand why.

P: It's more

P: to why not in a different way?

P: Like, why do I feel like that?

P: It's more, why not feeling in another way?

P: What's avoid me to feel differently in a positive way, let's say.

P: So it doesn't matter for you to know

P: you have daddy issues that if this don't solve the relation you have with your dad and poor dads maybe he's right and he's just being in a wall of shame because he did some mistake you know so um this is my one of my concerns

P: The other limitation we already shared in our conversation, it's about the relation, this therapeutic alliance.

P: Not all the therapists will work with all people.

P: For example,

P: I can be your therapist and we work together or maybe you don't like the way I work and then you go to another person.

P: It can be maybe something that it's really personal for the person.

P: It's being too general and not making the person feel special the way they are.

P: It takes from us, from the client, identity.

I: um anything else

P: no no i think it's good

I: um so i wanted to clarify a few things so the you said that sometimes people come in with some beliefs right that uh were formed in some way maybe from tiktok or from somewhere else right that is uh

I: maybe blocking their their progress right because with this belief they they come to an understanding oh i have maybe maybe you say i have adhd or i have anxiety disorder or i have daddy issues or whatever right um and this this might block them from actually getting into a normal life where they might try and fix these issues right um

I: So let's see.

I: So you said it's not about the why, but it's about the why not.

I: So as I understand it, what you mean is that sometimes people come up with their beliefs, but these beliefs are not always helpful in getting to a better position, right?

P: Yeah, because sometimes these beliefs that are seen on an AI platform or in digital sources,

P: they are validating primary beliefs.

I: Yeah.

P: Okay.

P: For example, I, I think my father, um, I think men, uh, cannot love me.

P: Yeah.

P: That's what this is because you have that issues.

P: This, this information doesn't solve your problem.

I: Yeah.

I: Okay.

P: So it puts even more, um,

P: weight on your primary belief.

P: So it's this tendency to validate beliefs that they can be not the best for the person.

I: Yeah, so it's sometimes good to have validation, right?

I: And say, okay, this is correct or it's valid that you have this feeling or this might be the cause of your beliefs, right?

I: And sometimes AI is correct, right?

I: In that way where the person saying, I have daddy issues and this and this and this, there might be some things that are correct,

I: But there might also be other things that might not be correct.

I: And also sometimes it's not important to look at, okay, these are the reasons or these are the things that is causing this, but maybe look at, okay, but how do you fix that?

I: And also sometimes you need to point out, hey, but maybe this one thing that you mentioned is unrelated or is not correct.

I: And therefore let's not confirm it, right?

I: Then we need to dispute that and say, hey,

I: where maybe AI will say, oh, yeah, that's because of this or because you had this problem when you were five years old or whatever.

I: Instead, sometimes a psychologist's task is to say, hey, how about maybe you did something wrong there or maybe something else happened or there's always different options, right?

I: And sometimes a psychologist can be more critical of their client and have a more critical view than maybe AI does.

P: yes and plus we don't say why we help we help the person to follow to follow in order to find their why yeah where when when happened when did you learn this yeah yeah and this is really transforming

P: This is the process, I'd say.

I: Okay, let's move on.

I: How are you with time, by the way?

I: Because we're already...

P: I have a session at...

P: 4.15, so we should finish at 4.10.

I: Okay, okay, okay.

I: But we'll finish before that.

I: We have like 15 minutes.

I: Yeah, I have two more questions, but these are usually not that long.

I: So the next question is about client perspective and therapeutic alliance.

I: How do you think clients respond to AI being used in their treatment?

P: How do I...

P: What do I think that clients think about using AI in the sessions?

P: There are a difference between using AI in the sessions and AI do sessions or do a familiar work.

P: But in the sessions, I think it would depend on age.

P: and problematic.

P: The reason for the process.

P: I think young people would be more...

P: They could accept better.

P: Also because they tend to have more facility

P: on using these tools, it could be more difficult with other people.

P: In general, when they use AI tools between sessions, they came with this information with some shy, like, I check on GPT, but he didn't say much.

P: It's like,

P: So I think if there is a way that I don't find of using AI during the sessions as a tool, I think it could be, people could accept because it would be already validated from the therapist.

P: So the therapist would choose this tool to work

P: as we use Google Teams or other things.

P: But first, I think it has to make sense to the therapists so we could introduce this tool like another tool.

I: Makes sense.

I: So you also mentioned that the age might be a factor, right?

I: Where young people might be more inclined to be open to it and older people might be less inclined to do that.

I: You also said that the problem case also depends or might affect the willingness of the clients, right?

I: So do you have any more information on that?

I: Like what type of people with what type of problems would be more open to it and what people might not be open to it?

P: people with anxiety issues, isolating the problematic because considering that it's an anxiety, a typical anxiety, right?

P: For this isolate problems,

P: people can be more reset, they can accept better.

P: But for example, for personality issues, it can be a big issue.

P: Because, for example, in some personality issues, there are people who don't trust enough for you to have

P: things in your computer or they ask only, they pay only in cash because they don't want to use the visa or, you know, so it can be in some pathologies can be, maybe it's not helpful.

I: Yeah.

P: Yeah.

P: And it's that.

I: Yeah.

I: So it's, um, you think in general it might work, but then for some people there is some, um, I just had the word, but now I forgot it.

I: Um, good question.

I: What is it?

I: Let me Google it fast.

I: Suspicion.

I: So there are some pathologies that have suspicious tendencies, right?

I: So maybe some people with psychotic symptoms or hallucinations or that kind of stuff.

I: they might have some tendency to be very suspicious of people or of anyone in particular right so they might also be suspicious of ai listening in or ai being used and some how their data is going to that thing right yeah i think we have to consider the facility of people using digital platforms in general um

P: Because I think the main point would be the therapists consider this tool in their sessions.

P: Yeah.

P: And how?

P: Because if we introduce, because there are a lot of tools and we select the tools that we might, we think that might work with the person and they still can, they can not work together.

P: even though you thought that they would.

P: So it could be a tool like that.

P: The therapist consider this tool because they are comfortable using this tool and still they could not work on the person.

P: So it has to start on the therapist.

I: Yeah, first of all, of course, the therapist has to have trust in this tool to have potential to work, right?

I: It might not work, but at least they think it could be a benefit for their client.

I: And that's the first step.

I: And then the second step will be from the therapist to introduce this to the client and then discuss, of course, with the client, okay, this is the reason why I think it might be helpful.

I: we will try it out or like however you communicate that and then you will see if the client approves of that or doesn't want to do that but it's it's dependent on do you believe in the product or the tool and then also on like maybe young people are more willing to do it maybe older people are less and maybe some people with specific problems might not tend to agree with it because they're suspicious of other things

I: Right?

I: Yeah.

I: Yeah.

I: One question that came up in my mind is, do you think high comorbidity would also be harder to introduce AI tools?

I: So, for example, if someone has anxiety and it's just anxiety, that's a bit more straightforward.

I: Maybe, of course, there's a lot of different anxiety disorders and different symptoms and whatever.

I: But when you introduce comorbidity, then it becomes, I think, harder, no?

I: Because then it's like, oh, is this the depression or is this the anxiety or is it a mix of both?

I: Like, for example, sleep could be depression or it could be anxiety or it could be even like a manic episode or it could be whatever, right?

P: Comorbidity...

P: Not always, but comorbidity can be related to personality structures.

I: Yeah, personality has a lot of comorbidity with a lot of things, right?

P: You never have a personality issue per se.

I: You always have something more.

P: With something, yeah.

P: So if we consider this thing that we told before about personality issues,

P: there are comorbidities with that so it could be could bring more resistance yeah

P: to use, but not only AI, any tools.

I: Yeah.

I: Yeah.

I: Yeah.

I: Yeah.

I: No, makes sense.

I: Um, okay.

I: Then this is the last question.

I: We have six more minutes, uh, professional identity, autonomy, and future conditions, which aspects of your work should remain strictly human.

I: And the second question is what would you need to feel confident about using AI safely in practice?

P: I would need to know for sure that there are security about the data and that the AI don't make decisions.

P: in the sense of diagnosis, suggestions of treatments in the way that it's the only one that works.

P: It could be useful if the AI consider scientific information

P: for example.

P: Yeah, like that.

I: Yeah.

I: Okay, makes sense.

I: And then the first part, so what, which aspects of your work should remain strictly human?

P: The therapy, the process, the therapeutic process.

I: Yeah.

P: yeah so why do you think these aspects need to stay human the therapeutic process because we are social humans and we need to um be in society yeah we need this belonging it's a biological need it's a it's an evolutive need

P: Yeah.

P: And in my view, we cannot have this with the AI tool.

I: Makes sense.

I: Okay, so that's it.

I: Do you have anything else to add or was that it?

P: No, I think the interview is long because I talk too much.

P: I know.

I: No, no, no.

I: I also did a talk in the beginning.

I: In general, it's nice that I have a lot of information, so that's always better.

I: So more talking is better.

P: If at some point you need answers to more questions because it's a qualitative research, I'm available to...

P: yeah to answer that's okay but for now i i have nothing more to say

I: that's good that's fine uh then i will stop the recording